

OREGON STATE HOSPITAL

POLICY

SECTION 6: Patient Care **POLICY: 6.005**

SUBJECT: Patient Death and Organ Donation

POINT PERSON: Chief of Medicine

APPROVED: Sara Walker, MD

DATE: AUGUST 8, 2024


Interim Superintendent

SELECT ONE: ☐ New policy ☐ Minor/technical revision of existing policy
☐ Reaffirmation of existing policy ☒ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Oregon State Hospital (OSH) recognizes that the death of a patient is a significant event which must be handled with sensitivity and respect. This policy sets forth guidance for managing the event, notifying appropriate parties, investigating certain situations, reporting the event, organ donation processes, and disposition of the deceased patient's remains and possessions. OSH will manage the event as delineated in this policy and related procedures.
- B. This policy applies to all OSH staff and patients.

II. POLICY

- A. Patient Death
 - 1. Before an anticipated patient death, the patient's treatment team must ask the patient if they consent to information about their death being shared with OSH staff and patients, and whether they would like a memorial service at OSH. Staff must inform the Spiritual Care Director and Program Director via email of the patient's choices.
 - 2. If able, staff must attempt to acquire funeral arrangement information in instances of anticipated deaths.
 - a. If the deceased person is indigent, OSH will work with a local mortuary and assist with the disposition of the patient's remains in the least costly, most environmentally sound manner, that does not conflict with known wishes of the deceased and that complies with law.

- b. In such cases, the patient's family is not able to direct the location and manner of the disposition of the remains.
- 3. The deceased patient maintains protected health information privacy rights as provided in the Health Insurance Portability and Accountability Act. All communications and processing of their medical record must comply with applicable OSH procedures and state and federal regulations.
- 4. Notification of the next of kin and proper authorities must be performed in accordance with Oregon Revised Statutes (ORS) and established policies and procedures.
- 5. After initial notification of the next of kin or guardian has been completed, only the Chief Medical Officer or designee, the Spiritual Care Director or designee, the Program Director, OSH Security or OSH Ombuds may communicate further information with the family.
- 6. Unless the Superintendent expressly permits, only the Security Director or designee may call the Oregon State Police and Medical Examiner. All investigations must be handled as directed by applicable OSH policies and state laws.
- 7. The Superintendent must retain and submit reports to external agencies as required by ORS 179.509 and ORS 432.083.
- 8. If a death meets sentinel event criteria indicated in OSH Policy 2.012, "Sentinel Events," the death must be reported as indicated in that policy.
- 9. In the event of any patient death, staff must follow Procedures A.
- 10. If deemed necessary for an unusual death, such as a death that occurs outside of OSH, and in cases of medical, legal, or educational interest, the Superintendent may request the medical examiner conduct an autopsy in accordance with requirements of ORS 97.082. The request must be documented on the medical examiner's records request form (located on the I: Drive).
- 11. Transport of the remains must occur as soon as possible as delineated in ORS chapter 97 after the release of the body by the Medical Examiner and Oregon State Police or, if no Medical Examiner or OSP involvement, after confirming with next of kin which funeral home they designate.

B. Organ Donation

- 1. Because the donation of internal organs requires a patient to be ventilated, only tissue and eye donations after cardiac death are possible for patient deaths that occur on the OSH campus. OSH uses organ procurement organizations (OPO) to coordinate the donation of tissues and eyes.
- 2. OSH does not engage in the acquisition, receipt, storage, or issuance of organs, tissues, or eyes.
- 3. In accordance with ORS and OAR, only trained designated requestors may approach families about donation. For OSH, the trained requestor is the OPO.

- C. The deceased patient's property must be disposed of per procedures attached to this policy; OSH Policy 8.037, "Patient Property"; and other relevant state regulations.
 - 1. If the patient passes and OSH staff cannot contact next of kin, staff must send patient property with the remains to the funeral home, other items staff are unable to send shall remain in long term storage and be handled per OSH policy 8.037 "Patient Property."
- D. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- E. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

"Indigent person" means a deceased person who does not have a death or final expense benefit or insurance policy that pays for disposition of the deceased person's body or other means to pay for disposition of the deceased person's body and:

- a. Who does not have a relative or other person with the legal right to direct and the means to pay for disposition of the deceased person's body;
- b. Whose relative, or other person, with the legal right to direct the disposition of the deceased person's body does not pay or arrange to pay for, or refuses to direct, the disposition of the deceased person's body within 10 days of being notified of the death; or
- c. For whom no person other than a person described in paragraph (a) or (b) of this subsection wishes to direct and pay for the disposition of the deceased person's body.

"Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

IV. PROCEDURES

Procedures A Patient Death Response Process

V. ATTACHMENTS

Attachment A Patient Death Checklist

VI. RELATED OSH POLICIES AND PROTOCOLS

- 1.003 Incident reporting
- 1.013 Data governance
- 2.012 Sentinel events
- 2.014 Medical record transportation and maintenance
- 5.024 HEART Trauma response program
- 6.003 Seclusion and Restraint
- 6.012 Code Status
- 6.025 Advance directives
- 6.045 Clinical documentation
- 6.052 Trauma screening
- 8.019 Staff response to alleged criminal acts and contraband
- 8.037 Patient property and valuables: Handling and storage
- 8.038 Code Blue medical emergency

VII. REFERENCES

42 CFR § 482.13

42 CFR § 482.45.

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for hospital*, PC.03.05.15. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for hospital*, PC.03.05.19. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for hospital*, RI.01.05.01. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for hospital*, MS.05.01.01. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for hospital*, TS.01.01.01. Oakbrook Terrace, IL: Author.

Oregon Administrative Rule § 309-033-0733.

Oregon Administrative Rules §§ 333-505-0090 — 333-505-0110.

Oregon Administrative Rules § 407-014-0020.

Oregon Revised Statute § 97.082.

Oregon Revised Statute § 97.130.

Oregon Revised Statute § 97.160.

Oregon Revised Statute § 97.170.

Oregon Revised Statute § 97.190.

Oregon Revised Statutes §§ 97.951 — 97.985.

Oregon Revised Statute § 179.509.

Oregon Revised Statute § 432.083.